

SPECIMENS:

- A. LIVER NODULE
- B. RIGHT COLON

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GROSS DESCRIPTION:

A. LIVER NODULE

Received fresh for frozen section is a tan pink soft tissue fragment 0.3cm. Toto FSA.

B. RIGHT COLON

Received is a segment of right colon measuring 19.0 cm long with average circumference of 7.0 cm. Its proximal end is still attached to the terminal ileum (4 cm long with a 3.0 cm circumference). The appendix is 6 cm long with a 0.6 cm diameter. There is a cecal mucosa based mass, 7cm from the terminal ileum margin and 11 cm from the distal colonic margin. Both margins are free of tumor or any additional findings. The mass is exophytic, tan color, with a rolled edge, which is sharply defined from the adjacent mucosa. The mass is also circular and measures 4.0 cm in diameter and < 1 cm in depth (limited to muscularis propria on cross sectioning). On the radial aspect, black ink was applied. The mesocolon attached is grossly unremarkable, and measures approximately 1-5 cm in thickness. The remaining ileal and colonic mucosa shows no significant pathology. Summary of blocks:

B1: terminal ileum margin

B2: distal colon margin

B3-B10: tumor

B11: appendix with bisected tips and complete cross section and small lymph nodes

B12: contains fat nearest to the tumor invasion area, which is inked black –
also contains lymph nodes

B13-B20: possible nodes

DIAGNOSIS:

A. LIVER, NODULE, BIOPSY:

- MINIMALLY ATYPICAL EPITHELIOID CELL PROLIFERATION.
- SEE COMMENT
- NEGATIVE FOR COLORECTAL CARCINOMA

B. RIGHT COLON, HEMICOLECTOMY:

- INVASIVE, MODERATELY DIFFERENTIATED ADENOCARCINOMA WITH FOCAL MUCINOUS FEATURES.
 - TUMOR INVADES INTO SUBSEROUSAL TISSUE
 - SURGICAL MARGINS ARE FREE OF TUMOR
 - SEE TEMPLATE
- FIFTEEN LYMPH NODES NEGATIVE FOR CARCINOMA (0/15)

COMMENT: The liver nodule shows a proliferation of relatively bland epithelioid cells separated by a fibrovascular stroma. Unfortunately, the tissue is largely absent on permanent sections and can only be seen well on the frozen section. Ancillary studies like IHC are not possible. The epithelioid cells may represent histiocytes or potentially a hyperplastic mesothelial proliferation. There is no evidence of carcinoma, and I favor a benign process in this nodule. Correlation with clinical findings is recommended.

Microsatellite instability testing has been ordered on the colon tumor. Those results will be issued in a separate report.

SYNOPTIC REPORT - COLON & RECTUM

Specimens Involved

Specimens: B: RIGHT COLON

Specimen Type: Right hemicolectomy

Tumor Site: Cecum

Tumor Configuration: Exophytic (polypoid)

Infiltrative

Tumor size: 4cm

WHO Classification

Adenocarcinoma 8140/3

Histologic Grade: G2: Moderately differentiated

Extent of Invasion: Subserosa

Margins: Margin(s) uninvolved by invasive carcinoma (Proximal, Distal, Radial)

Distance of invasive carcinoma from closest margin: 7cm

Margin: Proximal

Venous/Lymphatic Invasion: Absent

Perineural Invasion: Absent

Additional Pathologic Findings: None identified

Extent of Resection: R0: Complete resection with grossly and microscopically negative margins

Lymph Nodes: Negative 0 / 15

Implants: Absent

Pathological Staging (pTNM): pT 3 a/b N 0 M X

CLINICAL HISTORY:

None provided.

PRE-OPERATIVE DIAGNOSIS:

Colon carcinoma.

INTRAOPERATIVE CONSULTATION:

FSA: No evidence of metastatic adenocarcinoma proliferation of atypical epithelioid cells and vessels.

Favor benign lesion. Defer to permanent. This case was reviewed by Dr. who concurs with the diagnosis. [REDACTED].

ADDENDUM:

SUMMARY OF IMMUNOHISTOCHEMISTRY/SPECIAL STAINS

Material: Specimen B

Population: Tumor Cells

Stain/Marker:Result: Comment:

MLH1 Positive

MSH2 Positive

PMS2 Negative

MSH6 Positive

The interpretation of the above immunohistochemistry stain or stains is guided by published results in the medical literature, provided package information from the manufacturer and by internal review of staining performance and assay validation within the Immunohistochemistry Laboratory of the. The use of one or more reagents in the above tests is regulated as an analyte specific reagent (ASR). These tests were developed and their performance characteristic determined by the Department of Pathology Laboratory at. They have not been cleared or approved by the U.S. Food and Drug Administration. The FDA has determined that such clearance or approval is not necessary.

Special stains and/or immunohistochemical stains were performed with appropriately stained positive and negative controls.

INTERPRETATION: No PMS2 expression identified.

The results of microsatellite instability testing will be issued as a separate report.

[REDACTED]

[REDACTED] [REDACTED]
[REDACTED] [REDACTED]
[REDACTED] [REDACTED]
[REDACTED] [REDACTED]